

Endoscopy Analysis Report

Case ID	CASE_19B22361	Report Date	2026-08-19T21:58:45
Detection Model	best.pt	Inference Device	cpu
Total Findings	1	Image Size	640 x 640

CV Findings (Structured)

Finding ID	Class	Confidence	Anatomical Location	Severity	BBox (x1,y1,x2,y2)
FND_70B6A8	Normal-Z-Line	0.90	gastroesophageal junction	unknown	59.9, 3.7, 422.2, 482.1

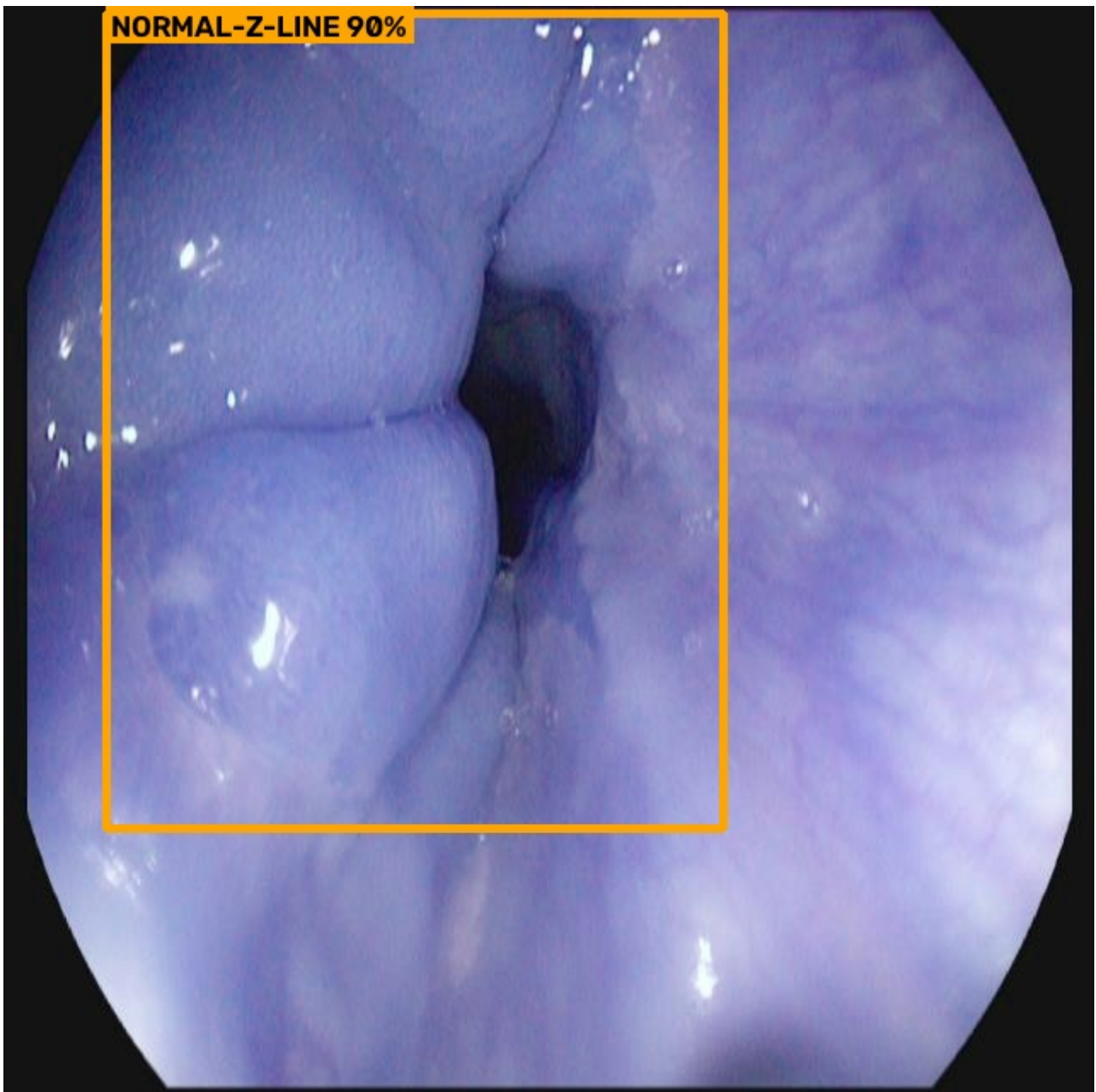


Fig. 1 - Annotated frame with CV model detections (bounding boxes and class labels).

Clinical Report

Capsule Endoscopy Report – CASE_19B22361

Date of Examination: 2026-08-19

Device: Capsule Endoscope (CPU-based analysis)

Model: best.pt

Summary

The analyzed frame demonstrates a normal gastroesophageal junction (Z-line) with no additional mucosal abnormalities detected. The Z-line appears intact and well-demarcated, suggesting no evidence of

reflux-related mucosal injury or Barrett's esophagus in this single frame.

Findings

Finding ID	Class	Confidence	Anatomical Location	Severity
FND_70B6A8	NormalZLine	0.898	Gastroesophageal junction	Unknown

Uncertainty: Severity is not specified; no other lesions were identified in this frame.

Clinical Impression

The capsule endoscopy frame shows a normal Zline at the gastroesophageal junction. No abnormal findings were detected. Further frames should be reviewed to confirm the overall mucosal status, but based on this image alone, no additional diagnostic or therapeutic action is warranted.

Recommendations

Clinical Recommendations for CASE_19B22361

Priority	Recommendation	Rationale
Immediate	None	The capsule image shows a normal gastroesophageal junction (Zline) with no mucosal injury or Barrett's esophagus. No urgent intervention is required.
Short-term	1. Review the complete capsule video to confirm that no additional lesions (e.g., erosions, ulcers, or Barrett's segments) are missed in other frames. 2. Correlate findings with the patient's clinical history (e.g., heartburn, dysphagia, weight loss). If symptoms persist or worsen, consider a diagnostic upper endoscopy (EGD) for direct visualization and biopsy.	Evidence from systematic reviews shows that a single normal frame does not rule out pathology; full video review improves diagnostic yield. Symptom-driven endoscopy is recommended when clinical suspicion remains high.

Routine	1. Maintain standard GERD management (e.g., proton pump inhibitor therapy, lifestyle modifications) if the patient has a history of reflux symptoms. 2. Schedule routine follow-up (e.g., 6–12 months) if the patient is at risk for Barrett's esophagus (age >50, chronic GERD, male). 3. Educate the patient on red flag symptoms (odynophagia, hematemesis, unexplained weight loss) that warrant immediate evaluation.	Current guidelines (e.g., ACG 2023) recommend continued GERD therapy and surveillance in high-risk patients, even when initial imaging is normal. Patient education reduces delayed presentation of complications.
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Key Takeaway:

The capsule endoscopy shows a normal Z-line; no immediate action is needed. Ensure a full review of the study, correlate with symptoms, and continue routine GERD care and surveillance as indicated.

Consistency Review

Audit Review

Item	Observation	Verdict
Findings Omitted	All findings in the JSON (1 Normal Z-Line) are present in the report.	■
Confidence Values	Report lists 0.898 (rounded from 0.8979).	■
Anatomical Location & Severity	Both report and JSON specify gastroesophageal junction and “unknown” severity.	■
Total Findings Count	JSON total_findings: 1; report lists 1 finding.	■

Recommendations Support	Recommendations (no immediate action, full video review, symptom correlation, routine GERD management) are fully justified by the single normal finding.	■
Inconsistencies	None detected.	■

Verdict: PASS

Disclaimer. This report was generated by an AI-assisted decision-support system for evaluation purposes only. It is not a substitute for a physician's clinical judgement. Findings must be confirmed by a qualified endoscopist. This document is confidential and proprietary to CAPCELL-AI.

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